

23 Feb 2026

Department of Health and Human Services

Subject: Response to HHS Health Sector AI RFI - Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Docket HHS-ONC-2026-0001

Dear Secretary Kennedy, Department of Health and Human Services:

Thank you for the opportunity to comment on your request for information on Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

Harrison.ai is a global healthcare technology company enhancing clinician capacity and patient care through AI automation. Clinician-led and patient-first, our suite of comprehensive solutions supports earlier, more accurate radiologic diagnoses and seamlessly integrate into clinical workflows. Harrison.ai solutions are cleared in 40+ countries and available to half of all Australian radiologists. They are clinically deployed at 1,000+ customer sites globally, including 40+ NHS Trusts. With 3,400+ clinicians using our tools, Harrison.ai has impacted 6.7 million patients' lives to date. With a growing footprint across the United States, including 8 FDA clearances covering 12 chest x-ray and CT brain indications, 3 FDA Breakthrough Designations, and NTAP reimbursement, Harrison.ai is committed to supporting innovation, quality, and safety for American healthcare.

We have separately submitted a Citizen Petition (<https://www.regulations.gov/document/FDA-2025-P-5560-0001>) asking the Food and Drug Administration (FDA) to create a streamlined pathway to market for certain AI-powered radiology diagnostic devices. Under the new pathway, manufacturers that have already received 510(k) clearance could comply with robust post-market study requirements instead of submitting new 510(k)s for expanded capabilities. The proposed pathway would reduce delays and costs that restrict U.S. access to AI compared to other nations, with adequate controls to ensure continued safety and effectiveness. Harrison.ai shares the Administration's interest in ensuring American patients benefit from AI advances in healthcare that are already available in the EU, Asia Pacific, and elsewhere around the world.

While limited to certain radiology products, this proposal could have a substantial effect on promoting AI innovation, as the affected product codes represent ~15% of clearances on FDA's list of [Artificial Intelligence-Enabled Medical Devices](#). This targeted policy proposal will foster innovation, promote smart regulation that could lower costs for manufacturers, healthcare providers and patients, and facilitate greater access to advanced AI healthcare technology, all while maintaining the US gold standard for safety, effectiveness, and quality.

We wish to take this opportunity to summarize below how our proposal is relevant to the following 3 questions from the RFI:

Specific Question #1: What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The FDA's current regulatory pathway is impeding access to the full benefits of AI-powered diagnostic devices in radiology and other clinical disciplines. Compared to other leading jurisdictions for AI development, the U.S. regulatory pathway is burdensome and ill-suited to the rapidly evolving nature of AI. This has resulted in a demonstrable AI innovation gap in healthcare compared to competitor nations, a corollary gap in U.S. patients' access to diagnostic tools in radiology that are at the leading edge of healthcare, exacerbation of existing physician shortages in these areas, and pricing pressures that widen the gap in innovation and patient access.

Manufacturers are forced to focus on applications with obvious return on investment due to the high cost of 510(k) clearance, coupled with unclear reimbursement pathways. Manufacturers often significantly limit their products' full offering for the U.S. market specifically, and AI diagnostic findings available in the US are vastly fewer than in the EU, UK, and Asia-Pacific. This restricts their broader adoption and utilization across healthcare systems, preventing patients and providers from fully benefiting from these technologies.

For evidence and examples of this innovation gap summarized above, please see our response to Question #6 below and also Section I in the petition:

<https://www.regulations.gov/document/FDA-2025-P-5560-0001>

Specific Question #2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

We recognize and commend the FDA's continued commitment to updating its regulatory frameworks for software-driven medical technologies. In reviewing potential mechanisms to support broader indications, we examined whether current tools could fulfill the same role as a more targeted exemption. Based on our analysis, we do not believe they can at this time.

Although frameworks such as Predetermined Change Control Plans (PCCPs) have offered value in specific situations, they are not ideally designed to facilitate substantive indications expansion in their current form. Even with expanded parameters, the requirement for comprehensive, up-front review of study methodologies, data sources, and acceptance metrics for each new category would likely replicate many burdens of the current framework. Our experience indicates that even minor updates often necessitate exhaustive pre-specification that can lag behind real-world product development cycles. A limited exemption pathway, anchored by postmarket evidence, would better promote innovation while maintaining safety.

Section 3054 of the 21st Century Cures Act amended sections 510(l) and 510(m) of the FD&C Act. As amended, these provisions require the FDA to identify within a certain timeframe and through publication in the Federal Register, any type of class I or class II device, respectively, that the FDA determines no longer requires a submission under section 510(k) of the FD&C Act to provide reasonable assurance of safety and effectiveness. (<https://www.fda.gov/medical-devices/classify-your-medical-device/class-i-and-class-ii-device-exemptions>)

While FDA has worked hard to fulfill its mandatory obligations, we believe the existing policy framework of a 510(k) exemption can be leveraged more to increase AI innovation, while still imposing limitations to ensure safe, effective devices on the market.

When warranted as part of the exemption process, FDA can also impose limitations on the exemption. For example, under 21 C.F.R. § 865.5950, the FDA requires a 510(k) before a manufacturer markets a genetic health risk assessment test but may expand the indications for the test, within certain limitations, without seeking a new 510(k). For that test, although the FDA determined that “a one-time FDA review (*e.g.*, premarket notification) [was] necessary to provide reasonable assurance of the safety and effectiveness of the device,” subsequent premarket notifications for certain changes were unnecessary.

As part of the exemption process, FDA can also impose other limitations in the form of special controls. Under our proposal for certain radiology devices, the new pathway would only be available to manufacturers of computer aided detection and/or diagnosis (CAD) and computer aided triage and notification (CADt) devices with at least one prior FDA 510(k) clearance under a relevant regulation. To be eligible for this “partially exempt” pathway, manufacturers would need to comply with new controls establishing active post-market monitoring, transparency, and mandatory training programs for qualified end users. All current CAD and CADt special controls would still apply. In addition, the partially exempt products would remain Class II medical devices that are subject to all other FDA regulatory oversight. This includes audit, quality management systems, complaint handling, adverse event reporting, recall management, and registration and listing requirements. The existing 510(k) pathway, with no changes to the special controls, will remain available to manufacturers who prefer it.

The subject of the petition (<https://www.regulations.gov/document/FDA-2025-P-5560-0001>) are the following regulations:

- “Computer-Assisted Diagnostic Software For Lesions Suspicious For Cancer (CADx)” (§892.2060, Product Code POK)
- “Analyzer, Medical Image (CADE)” (§892.2070, Product Code MYN)
- “Radiological Computer Assisted Detection/Diagnosis Software for Fracture (CADE/x)” (§892.2090, Product Codes QBS, QDQ)
- “Radiological Computer-Assisted Triage and Notification software (CADt)” (§892.2080, Product Codes QAS, QFM)

The high cost of 510(k) clearance, coupled with unclear reimbursement pathways, forces manufacturers to focus solely on applications with obvious return on investment. This financial pressure limits innovation and exploration of diverse use cases. Lowering market access costs would allow manufacturers to pursue broader findings and use cases, which could serve a larger

group of patients, while reducing the need for high pricing strategies to offset development expenses.

CAD devices in radiology typically receive 510(k) clearance for 1-2 findings per submission. Manufacturers seeking approval for a larger set of findings usually need to split these into multiple submissions in order for them to be suitable for 510(k) review. In radiology, where comprehensive image review is crucial, supporting a more holistic approach might require clearing 100-1,000+ findings, depending on the specific application. As described in more detail in the petition, a 100+ finding device that can take one submission and 6 months to gain clearance in Europe may involve multiple submissions, taking a decade or more and tens of millions of dollars, which simply does not lead to favorable return on investment to pursue.

Even more important than the number of submissions is consideration of the advantages and disadvantages of more complex study designs, as shown in the example in the figure below. While device manufacturers should (and do) conduct safety and effectiveness testing before placing a device on the market, the FDA's current regulatory paradigm for CAD devices involves conducting studies on a narrow finding basis. While EU and other outside US regulators routinely accept clinical studies with many findings, to meet all of FDA's expectations, manufacturers often choose studies that focus on a limited set of findings, which can involve a physician searching for a single type of abnormality. In contrast, in clinical practice, physicians review an entire image, searching for many findings at the same time. A study design that involves searching for one or only a few findings has the advantage of being cleaner from a statistical point of view but a multiple finding study has the advantage of being more representative of clinical practice and enabling a lower cost, less burdensome path to market.

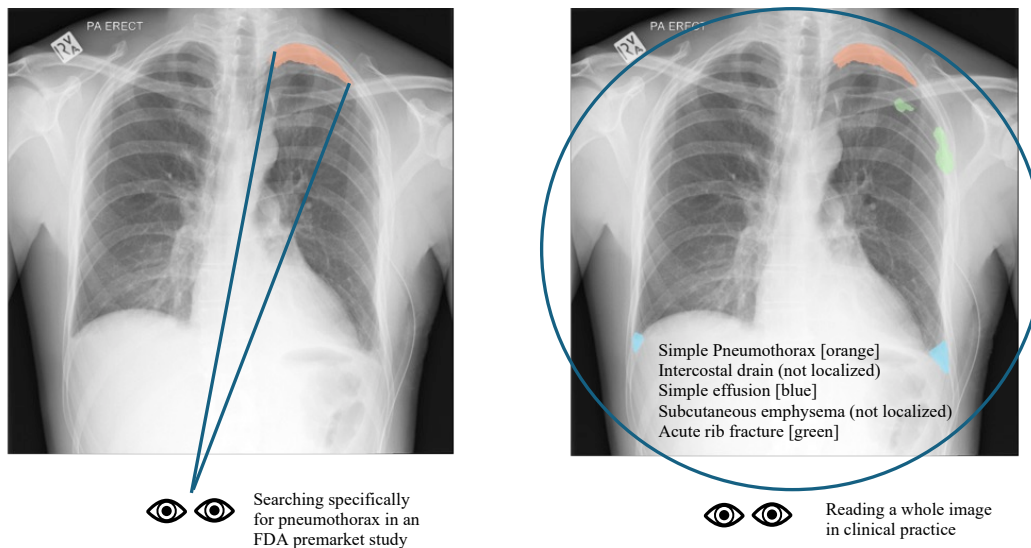


Figure 1: To address all of FDA's statistical questions, manufacturers often choose single finding study designs (at left), although in clinical practice physicians review an entire image (right) and so a multifinding study design is arguably more reflective of clinical practice. The chest x-rays at left and right are identical, and yet in the single finding study pictured on the left, the clinician would be focused only on looking for pneumothorax.

Currently, the FDA expects manufacturers of CAD (CADE, CADx, and CADE/x) devices to fulfil the special control of “improved reader performance” in the regulations listed above demonstrated through multi-reader, multi-case (MRMC) studies. For CADt devices, however, the FDA accepts standalone studies. Standalone studies compare AI output against a reference standard, or “ground truth,” often established by panels of experienced radiologists. By contrast, reader studies assess the performance of clinicians, including less experienced radiologists, both with and without AI assistance, relative to the same reference standard. Although reader studies can demonstrate improvements in clinical decision-making, they are resource-intensive, requiring multiple clinician reads per case and lengthy “washout periods” between AI-assisted and unassisted readings. They may cost more than five times that of standalone evaluations and, in addition to being complex to design, often suffer from limited generalizability to real-world practice.

This policy divergence between premarket expectations for CAD and CADt devices has unintentionally incentivized manufacturers to develop and market FDA-cleared systems with reduced functionality relative to their non-U.S. counterparts. Specifically, CADt devices are subject to a less burdensome premarket process evaluated through standalone studies, creating an incentive for developers to pursue indications for triage over indications for detection and diagnosis. But CADt devices do not typically support explainable AI outputs, such as abnormality localization, despite the latter’s widely recognized role as a risk mitigator, making the indication a less powerful tool for clinicians. Importantly, other regulatory authorities have accepted standalone performance studies, supplemented by systematic analysis of the scientific literature, as sufficient justification for claims of benefit in improving reader performance. Although FDA has traditionally recommended MRMC studies, the existing special controls for CADE, CADx, and CADE/x are written broadly enough to accommodate alternative forms of evidence.

By moving some evaluation to post-market, the exemption proposal will allow manufacturers more flexibility in picking appropriate, least burdensome testing methodologies that still satisfy the special controls. For example, manufacturers may choose to validate their technologies using multi-finding studies that are more reflective of clinical practice, as in the example in the figure above.

Due to the considerable costs associated with device clearance and reimbursement uncertainty, manufacturers are compelled to seek high prices for their AI solutions. This pricing strategy can limit adoption and accessibility, particularly for smaller healthcare providers or resource-constrained settings, including rural, low-income, and community hospitals. Without reimbursement, pricing is typically constrained by the interpretation fee for reading an exam. For chest x-ray, the interpretation fee is ~\$10 and typically providers may only be willing to share a small fraction with the AI company, no matter how comprehensive the product.

Therefore, it is also important to consider other federal policies that hinder AI adoption, importantly restrictions and lack of clarity in reimbursement pathways for AI-enabled software solutions, which force manufacturers to focus on applications with obvious return on investment, resulting in limited offerings and reduced access for American patients.

Specific Question #6: Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Examples of impactful Harrison.ai tools broadly utilized outside the US

The most impactful AI tools are those that are explicitly designed around real-world workflow efficiencies rather than isolated algorithmic performance metrics. For example, our comprehensive chest x-ray AI, which detects 100+ radiological abnormalities, has been deployed across more than 45 NHS trusts (representing ~one-third of all UK chest x-rays), offering a model for overburdened US systems. A single comprehensive platform avoids the cost and complexity of integrating and training physicians on multiple point solutions, and current FDA-cleared CXR tools, even combined, still fall well short of our abnormality coverage.

Comprehensive AI enables multiple workflow efficiency use cases, and unfortunately, our best examples are not in the US. In the UK, our platform helps prioritize urgent CT brain cases and could cut overnight teleradiology outsourcing by up to 70% (simulation-based), translating to hundreds of thousands of dollars in potential annual savings for a comparable US hospital, with no equivalent FDA-cleared capability today. NHS trusts are also exploring use cases where only likely “remarkable” or “abnormal” exams are routed to overstretched subspecialists. Looking forward, Harrison and other companies are working toward even more comprehensive tools that could materially reduce reporting time, an area where ex-US pathways exist but FDA clearance remains very challenging.

For chronic conditions, UK customers have focused on improving detection of incidental lung nodules and enabling follow-up CT before patients leave the hospital. This work is expanding to COPD and other underreported chronic diseases, which is highly relevant to the US, where COPD causes about 140,000 deaths annually and rural communities face a 70% higher COPD death rate than the national average (CDC Urban-Rural County and State Differences in Chronic Obstructive Pulmonary Disease, <https://www.cdc.gov/mmwr/volumes/67/wr/mm6707a1.htm>; American Lung Association COPD Trends Brief – Burden; (<https://www.lung.org/research/trends-in-lung-disease/copd-trends-brief/copd-burden>)).

US data with >3700 mixed inpatient and outpatient chest X-ray cases from Mass General Brigham and >1200 outpatient cases from Premier Radiology Associates show our AI can catch clinically important “confirmed misses” related to chronic conditions: 5-8% for osteopenia related findings, ~20% for cardiomegaly (related to CHF), up to ~11% for COPD-related findings, ~7% for pneumonia, and ~2% for lung nodules (data on file; portions of results also presented at RSNA).

Despite strong evidence (including analyses of US data for both cleared and uncleared findings) with minimal adverse events across millions of cases processed worldwide, current FDA pathways are too burdensome, so these comprehensive capabilities remain largely inaccessible to US healthcare providers.

Broader radiology industry trends

The above company-specific example of the clinical impact of comprehensive AI capability that is not available to US providers and patients is just one example of the broader US AI innovation gap across the industry.

Evidence of a US AI innovation gap in diagnostic radiology has previously been publicly reported:

- A comparison of three manufacturers' clearances for chest radiography in the U.S. and Europe showed the following: 124 findings cleared in EU vs. 5 in U.S. for manufacturer 1; 10 findings cleared in EU vs. 2 in U.S. for manufacturer 2; and 15 findings cleared in EU vs. 1 in U.S. for manufacturer 3. In addition, the EU versions of these devices offer greater transparency and explainability through localization of findings and heatmaps whereas the U.S. versions do not. (Hillis JM, V. J. 2024, Mar 15. The lucent yet opaque challenge of regulating artificial intelligence in radiology. *NPJ Digit Med*, 7(1). doi:10.1038/s41746-024-01071-2)
- Presentation at Nov 2024 FDA Digital Health Advisory Committee meeting (excerpt below)

REGULATORY (SAMD)

THE ERA OF NARROW AI

The AI coverage of diagnostic tasks in the US is about 1%

Potential AI Findings	Modalities	Computed Tomography	Magnetic Resonance	Nuclear Medicine	Radiography/Mammography	Angiography	Ultrasound	Interventional
Organ Systems	3,581 (42)	828 (9)	690 (3)	318	687 (27)	417	410 (3)	231
Abdominal	449 (2)	186	60 (2)	40	40	35	65	23
Breast	164 (10)	30	25 (1)	25	55 (8)	0	25 (1)	4
Cardiovascular	276 (1)	55	45	55	7	55	45 (1)	14
Emergency	364	105	35	2	105	45	55	17
Musculoskeletal	529 (4)	135	165	12	157 (4)	14	25	21
Neuroradiology	595 (5)	87 (3)	175	54	19 (2)	155	25	80
Pediatric Imaging	584 (6)	105	155	25	84 (6)	73	105	37
Thoracic Imaging	620 (14)	125 (6)	30	105	220 (7)	40	65 (1)	35

Chest Xray

The same AI products in UK, EU and Asia provide full diagnostic coverage

Figure 2: Difference between the number and nature of AI diagnostic findings available in the US (red) and UK, EU and Asia (black). Slide reproduced from Dr. Keith Dreyer's presentation to the FDA Digital Health Advisory Committee on Total Product Lifecycle Considerations for Generative Artificial Intelligence-Enabled Medical Devices, 20th November 2024 (Dreyer, 2024).

While the figure above provides a broad, generalized clinical view of the innovation gap across multiple manufacturers, our analysis below of AI radiology clearances in EU vs. US provides some additional key insights. The total number of radiology AI clearances do not paint a full picture of the differences in what is available US vs. ex-US: 297 for EU vs. 1039 for US FDA as of Jan 2026.

A more granular analysis of cross regional manufacturer patterns, focusing on CAD and CADt type devices which detect and/or or triage abnormalities, reveals a significant gap:

- 24 radiology AI manufacturers have at least one CE-marked CAD/CADt product and zero FDA clearances; 7 of these sell relatively comprehensive multi-finding CAD products in the EU.
- In contrast, only 11 manufacturers have at least one FDA CAD/CADt clearance and no CE mark, generally with narrower US indications; only one of these offers a relatively broad x-ray suite and one a multi-finding fetal ultrasound CAD product.

Because of their importance in addressing efficiency challenges posed by physician shortages, we also focused our analysis specifically on availability of “multi-finding” (5+) CAD and CADt type devices in EU vs US:

- 18 manufacturers have CE-marked multi-finding CAD/CADt radiology AI devices, typically with 5+ findings and often tens to >100 findings per modality, plus localization and richer reporting,
- Crosschecks with the FDA database indicate that US versions, where they exist, have fewer findings and fewer advanced features (e.g., limited or no lesion localization or comprehensive heatmaps, and less specificity on characterization of the abnormalities).
- Only 2 of these multi-finding manufacturers are US-based and 1 is no longer operating, supporting the hypothesis that US companies target narrower indications that fit within FDA constraints.
- In the US, multi-finding (5+ finding) CAD/CADt clearances are concentrated in a few pockets:
 - Musculoskeletal fracture tools with 10+ fracture “locations” that function more like subfindings,
 - Chest x-ray tools that flag “regions of interest” (i.e., bone, pleura) without enumerating specific abnormalities,
 - A small number of CT abdomen products with mid-single to low double-digit findings, and one fetal ultrasound CAD product.

The data presented for the EU (representing 27 member countries) is indicative of wider availability of diagnostic AI in many other regions. Multiple countries either recognize CE or MDR marking or provide expedited access pathways as a result of EU clearance. These include:

- Asia Pacific-Singapore, Malaysia, Australia, India
- Europe (non-EU)- Albania, Bosnia and Herzegovina, Montenegro, North Macedonia, Serbia, N. Ireland, Switzerland, Turkey, United Kingdom, Norway, Iceland, Liechtenstein
- Middle East- Israel, Saudi Arabia, Bahrain, Kuwait, Oman, Qatar, UAE

The analyses above utilized the following databases:

- Health AI Register (<https://healthairegister.com/radiology/products>), as of September 2025.
- FDA AI/ML Database: <https://www.fda.gov/medical-devices/software-medical-device-samd/artificial-intelligence-enabled-medical-devices>
- Note: *the focus was on **non-dental** AI devices in radiology*

The data presented here should be considered as providing directional insights, especially due to the limitations of incomplete product descriptions and mismatches in regulations and definitions across EU and US regulatory bodies.

Innovation gap extends to other clinical disciplines

While the focus of our response is diagnostic radiology, these US AI innovation gap trends are reported in other specialties. For example, a recent scoping review of regulator-approved ophthalmic image analysis AI devices found that 97% (35 of 36) were approved in the EU versus only 8% (3 of 36) in the United States [Ong AY, et al. A scoping review of artificial intelligence as a medical device for ophthalmic image analysis in Europe, Australia and America. NPJ Digit Med. 2025;8(1):323. doi: 10.1038/s41746-025-01726-8]. In dermatology, a comprehensive review identified 15 AI devices with regulatory approval globally, but only 3 FDA-approved systems, all hardware-based spectroscopy tools narrowly focused on melanoma and skin cancer [Nahm WJ, et al. Artificial Intelligence in Dermatology: A Comprehensive Review on Regulatory Approval in the United States and Internationally. Int J Dermatol. 2025 Sep 27. doi: 10.1111/ijd.17847]. The paucity of FDA cleared digital pathology AI compared with EU CE-marked tools has also been reported [Matthews, G.A, et al, Public evidence on AI products for digital pathology. NPJ Digit Med. 2024 Oct 25;7:300. doi: 10.1038/s41746-024-01294-3]. International platforms offer broader applications and condition-specific tools.

In order for US patients and providers to have access to some of the same AI innovations that are already available outside the US, we strongly urge the Administration to consider some of the regulatory reforms highlighted in this letter.

Thank you for your consideration and please address any questions to the email address associated with this electronic submission.

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